The Graveyard Shift: Survival Protocols

Triage, Delegation, & Bio-Hazard Containment

The pressure gauges are redlining. You are the Master Mechanist. Keep the machine running.

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PRIORITIZATION RULE #1: THERMODYNAMICS

ACUTE Beats CHRONIC

CHRONIC (Stable)

ACUTE (Unstable)

Example: Appendicitis (Acute) > COPD (Chronic).

A rusting hull can wait. A bursting pipe cannot.

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Prioritization Rule #2: The Fresh Assembly

- Surgery < 12 hours ago = PRIORITY
- General Anesthesia in first 12 hours = UNSTABLE

2-hour post-cholecystectomy (High Priority) > 2nd day post-coronary bypass (Low Priority).

FRESH POST-OP (<12 Hours)
Beats MEDICAL or OTHER SURGICAL

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STABLE (Safe)

Chronic
Post-op >12 hrs
Local/Regional Anesthesia
Lab values A or B
Ready for Discharge
Unchanged Assessment
Expected S/Sx

UNSTABLE (Danger)

Acute
Post-op <12 hrs
General Anesthesia (<12 hrs)
Lab values C or D
Newly Admitted
Changed Assessment
Unexpected S/Sx

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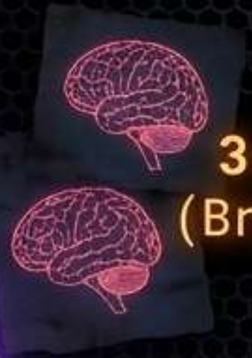
The Four Horsemen: Always Unstable

1. HEMORRHAGE
(Not just bleeding)

2. HIGH FEVER > 105°F
(Seizure Risk)



3. HYPOGLYCEMIA
(Brain Damage Risk)

4. PULSELESSNESS / BREATHLESSNESS

⚠ EXCEPTION: At unwitnessed accident, pulseless/breathless = Dead (Low Priority). ⚠

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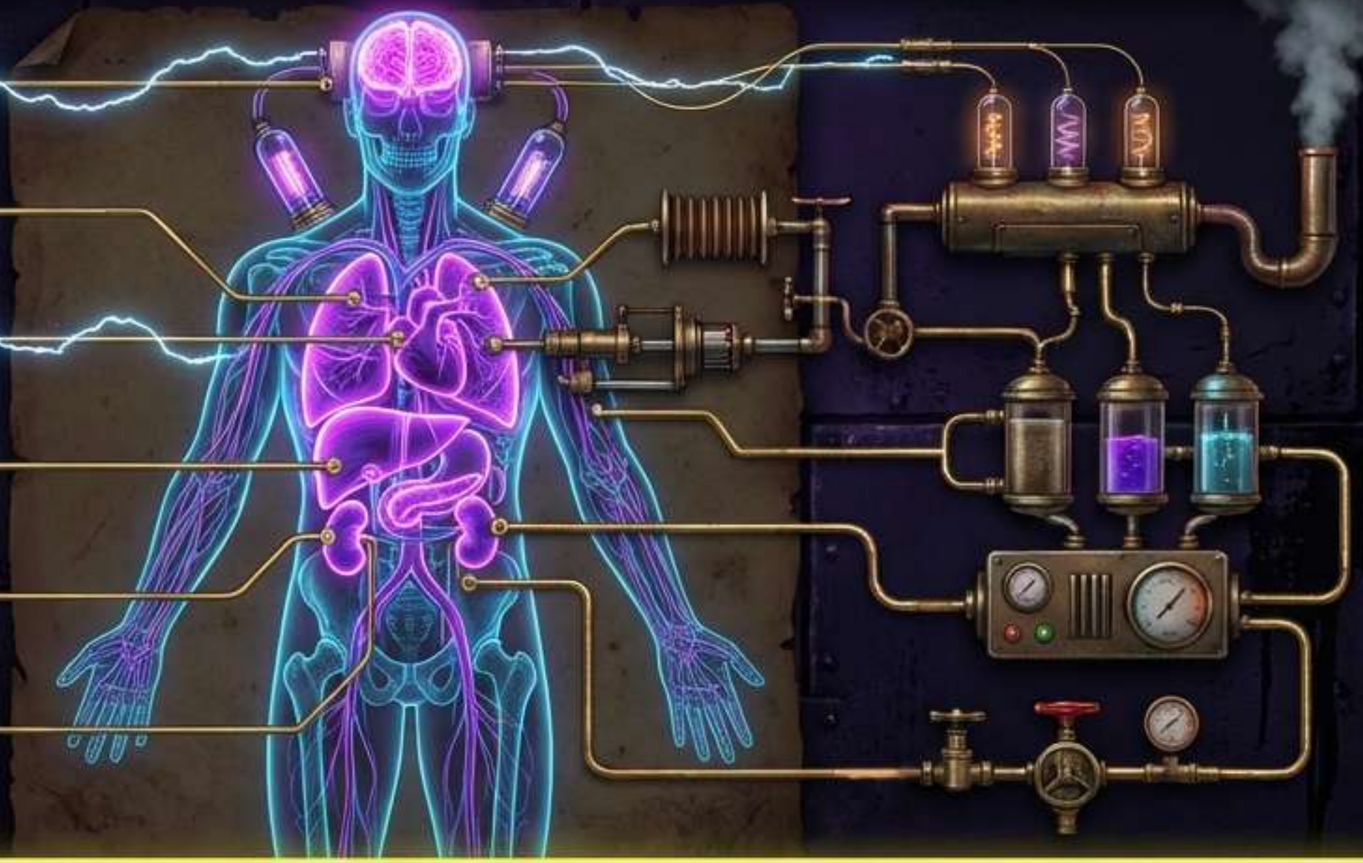
Prioritization Rule #4: The Vitality Engine

1. **BRAIN**
   (The Processor)
2. **LUNGS**
   (The Bellows)
3. **HEART**
   (The Pump)
4. **LIVER**
   (The Filter)
5. **KIDNEY**
   (The Waste System)
6. **PANCREAS**
   (The Regulator)



TIE-BREAKER: Use the organ in the MODIFYING PHRASE, not the diagnosis.

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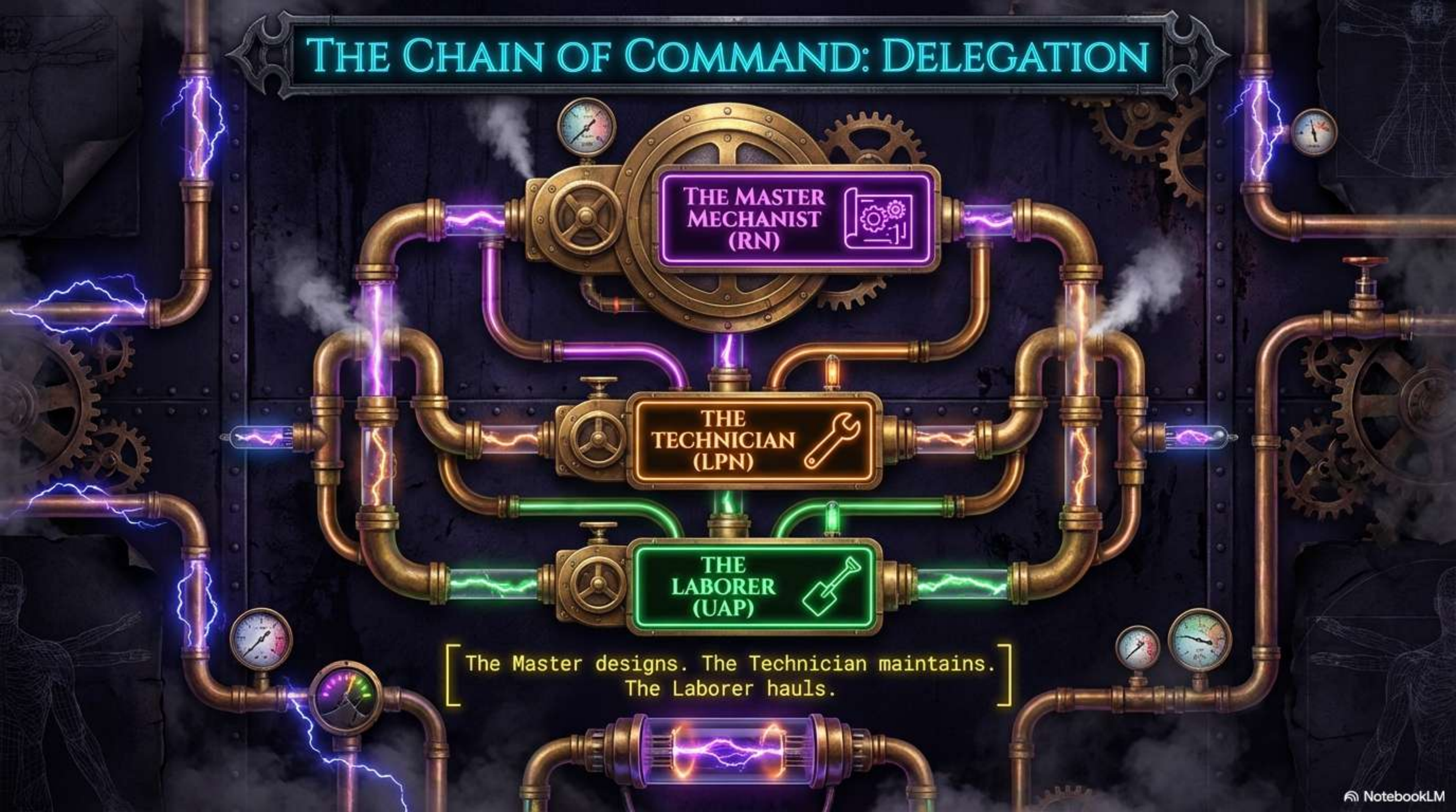
The Chain of Command: Delegation



[The Master designs. The Technician maintains. The Laborer hauls.]

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THE MASTER MECHANIST (RN)

Sole Responsibilities:

- **ASSESSMENT**: Admission, Discharge, Transfer, First after change.
- **PLANNING**: Creating the Care Plan.
- **TEACHING**: Initial teaching.
- **UNSTABLE PATIENTS**: Only the RN touches the unstable.
- **IVs**: Fluids, Blood Transfusions, and IV Push meds.

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THE TECHNICIAN (LPN)

- NO IV Push Meds
- NO Blood Administration
- NO Central Line Care (Flushing/Dressing)
- NO Planning (Implementation Only)
- NO Initial Teaching (Reinforcement Only)
- NO Unstable Patients

ALLOWED:

Stable patients, Tube feeds, Catheters, Meds (except IV push).

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THE LABORER (UAP)

ALLOWED TASKS

- ADLs: Baths, beds, ambulation (unless first time)
- Vitals: Routine measurement
- Accu-checks
- Enemas

PROHIBITED

- NO Medications (Exception: Topical OTC barrier creams)
- NO Assessments (Can chart actions, NOT patient status)

Chart "Side rail up". Do NOT chart "Patient tolerated well".

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QUELLING THE MUTINY: STAFF MANAGEMENT

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BIO-HAZARD CONTAINMENT PROTOCOLS

Seal the sectors. Do not let the miasma spread.

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Sector C: Contact Precautions

Enteric (C. diff, Hep A, Cholera)

Staph

RSV (transmitted via droplet, but Contact rules apply)

Herpes

The Armor

- GLOVES + GOWN
- Dedicated Equipment
- Private Room (Preferred)

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Sector D: Droplet Precautions

Meningitis

H. Flu
(Epiglottitis)

The Armor

- MASK + GLOVES
- Private Room (Preferred)
- Disposable Supplies
- Patient must wear mask when leaving room.

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SECTOR A: AIRBORNE PRECAUTIONS

M - Measles (MMR)

T - TB (Tuberculosis)

V - Varicella (Chickenpox)

THE ARMOR

- N95 Filter Mask (for TB)
- Negative Airflow Room
- Private Room (Required)

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Suit Up: Donning PPE

Reverse Alphabetical for G's, but Mask is 2nd

1. GOWN
2. MASK
3. GOGGLE
4. GLOVES

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DECONTAMINATION: DOFFING PPE

Alphabetical Order

1. GLOVES (Dirtiest)
2. GOGGLES
3. GOWN
4. MASK (Keep airway protected until last)

REFERENCE THE ORDER TO TAKE OFF PPE.

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MASS CASUALTY: THE BLACK TAG

Tag them black and ship them last.

1. Pulselessness
2. Breathlessness
3. Fixed and Dilated Pupils

Even if breathing, fixed pupils = Black Tag.

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SHIFT REPORT: SURVIVAL SUMMARY

- PRIORITIZE: Acute, <12hr Post-Op, Unstable.
- DELEGATE: RN plans/assesses. LPN maintains. UAP assists.
- MANAGE: Illegal? Supervisor. Danger? Confront.
- CONTAIN: Air (MTV), Droplet (Mask), Contact (Gown/Gloves).

The machine is stable. Shift complete.

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